

Report to **IMEDICAL,**
1 Union st
, Pyrmont, NSW, 2009

Patient **SCHLEGER, SHANE**
IMEDICAL C/O IMEDICAL 1 UNION ST PYRMONT NSW 2009

Phone 0415994541
D.O.B 01/01/1985 Age 40 years Sex M

Ref. by/copy to IMEDICAL, Collect date 05/03/2025 Lab ref 25-12699722
Collect time 11:22 AM Your ref
Reported 05/03/2025 09:12 PM

Tests requested SCP, AND, HOR, MBA, LIP, GLU, FBE

Clinical notes

SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)

Request Number 12699722
Date Collected 5 Mar 25
Time Collected 11:22
hsCRP (< 4.91) mg/L 0.93

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 10mg/L should prompt a search for a source of infection or inflammation.

SURGERY USE

Normal

No Action/File

Patient
Notified

Make
Appoint.

Further Tests

Notes
Required

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SERUM ANDROGENS

Total Testosterone (Siemens)	20.1	nmol/L	(8.3-29)
Sex Hormone Binding Globulin	44	nmol/L	(11-71)
Calculated Free Testosterone	348.8	pmol/L	(255.0-725.0)

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SERUM HORMONE PROFILE

Specimen Type: Serum								
Request	Date	FSH	LH	PROG	E2(ATEL)	E2(BECK)	LH/FSH	
Number	Collected	IU/L	IU/L	nmol/L	pmol/L	pmol/L	Ratio	
12699722	5 Mar 25				105			
Reference Ranges		FSH	LH	PROG	E2(ATEL)			
Males < 60 yrs		<7	<7	0.9-3.9	<150			
> 60 yrs		<14	<11	0.9-3.9	<150			

PLEASE NOTE:
'E2 (ATEL)' - Oestradiol by Siemens Atellica assay
'E2 (BECK)' - Oestradiol by Beckman Access assay

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SERUM CHEMISTRY

Specimen Type: Serum

Haemolysis	Nil		
Icterus	Nil		
Lipaemia	Nil		
Sodium	141	mmol/L	(135-145)
Potassium	4.6	mmol/L	(3.6-5.4)
Chloride	102	mmol/L	(95-110)
Bicarbonate	25	mmol/L	(22-32)
Anion Gap	19	mmol/L	(10-20)
Urea	7.2	mmol/L	(2.5-7.5)
Creatinine	100	umol/L	(60-110)
eGFR	83		mL/min/1.73sqM
Urate	0.40	mmol/L	(0.20-0.42)
Bilirubin	11	umol/L	(< 20)
AST	26	U/L	(< 40)
ALT	46	U/L	(< 40)
GGT	14	U/L	(< 50)
Alkaline Phosphatase	53	U/L	(35-110)
Protein	71	g/L	(60-82)
Albumin	46	g/L	(38-50)
Globulin	25	g/L	(20-38)
Calcium	2.45	mmol/L	(2.10-2.60)
Corrected Calcium	2.39	mmol/L	(2.10-2.60)
Phosphate	0.97	mmol/L	(0.75-1.50)
Magnesium	0.85	mmol/L	(0.70-1.10)

eGFR values between 60 and 89 mL/min/1.73m2 should be interpreted with caution. These results are only consistent with CKD in the presence of other evidence such as microalbuminuria, proteinuria or haematuria.
Ref:Lamb EJ etal in Ann Clin Biochem 2005; 42:321-345.

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LIPID STUDIES

Specimen Type: Serum

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis Nil
Icterus Nil
Lipaemia Nil

Fasting status	Fasting		
Total Cholesterol	4.4	mmol/L	(3.9-5.2)
Triglycerides	0.8	mmol/L	(0.5-1.7)
HDL Cholesterol	1.1	mmol/L	(1.0-2.0)
LDL Cholesterol	2.9	mmol/L	(1.5-3.4)
Non-HDL Cholesterol	3.3	mmol/L	(< 3.4)
Cholesterol/HDL-C Ratio	4.0		(< 5.0)

NVDPA TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL-C	>= 1.0
LDL-C	<2.0
NON HDL-C	<2.5

LDL-C exceeds target for higher risk patients and may be excessive in some individuals.

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SERUM/PLASMA GLUCOSE

Fasting status

Serum

Fasting

4.6 mmol/L

(3.4-5.4)

Normal glucose concentration.

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HAEMATOLOGY

Date Collected 05 Mar 25
Time Collected 11:22
Specimen Type: EDTA

Hb	156	g/L	(130-180)	WBC	5.7	x10 ⁹ /L	(4.0-11.0)
RCC	5.3	x10 ¹² /L	(4.5-6.5)	Neut	2.5	x10 ⁹ /L	(2.0-7.5)
Hct	0.46		(0.40-0.54)	Lymp	2.6	x10 ⁹ /L	(1.0-4.0)
MCV	86	fL	(79-99)	Mono	0.3	x10 ⁹ /L	(0.2-1.0)
MCH	29	pg	(27-34)	Eos	0.2	x10 ⁹ /L	(< 0.7)
MCHC	341	g/L	(320-360)	Baso	0.1	x10 ⁹ /L	(< 0.2)
RDW	12.4	%	(10.0-17.0)				
Plat	296	x10 ⁹ /L	(150-400)				

HAEMATOLOGY: FBC parameters are within reference range.